

Supplementary Appendix

Cost-Effectiveness of Intismeran Autogene (mRNA-4157/V940) plus Pembrolizumab versus Pembrolizumab Alone as Adjuvant Therapy for Resected Stage IIIB–IV Melanoma

Yichao Jin, PhD

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1 Model Structure and Assumptions

1.1 Health-State Definitions

Table S1: Health-State Definitions, Clinical Interpretation, and Model Inputs

State	Clinical Definition	Utility	Monthly Cost
RF	Recurrence-free after complete resection; receiving adjuvant pembrolizumab (up to 18 cycles) with or without ipilimumab (up to 9 doses)	0.83	\$0
LR	Confirmed locoregional recurrence without distant metastasis; includes salvage surgery, re-treatment, and surveillance	0.64	\$3,000
DM	Confirmed stage IV melanoma with distant metastasis; includes systemic therapy, palliative care, and monitoring	0.55	\$12,000
Death	All-cause mortality	0	\$0

Note: Utilities and costs are drawn from published melanoma CEA literature [1–3]. Costs are in 2026 USD. See Supplementary Table S9 for full PSA distribution parameters.

1.2 State Occupancy Equations

State occupancy at each monthly cycle is determined by the partitioned survival model (PSM) approach, using three parametric survival functions:

- **RFS(t)** — Recurrence-free survival: probability of remaining alive and free of any recurrence (locoregional or distant) at time t .
- **DMFS(t)** — Distant metastasis-free survival: probability of remaining alive and free of distant metastasis at time t .
- **OS(t)** — Overall survival: probability of being alive at time t .

The occupancy equations are derived from the ordering of these three events:

- All patients who are recurrence-free are also distant metastasis-free and alive: $RF = RFS$.
- Patients who have had a locoregional recurrence but have not yet developed distant metastasis are those who are distant metastasis-free minus those who are recurrence-free: $LR = DMFS - RFS$.
- Patients who are alive with distant metastasis are those who are alive minus those who are distant metastasis-free: $DM = OS - DMFS$.
- The dead state is the complement of overall survival: $Dead = 1 - OS$.

The constraint $RFS \leq DMFS \leq OS$ is enforced at all time points to ensure non-negative state occupancy. This ordering is clinically meaningful: a patient cannot have distant metastasis without having had a recurrence event ($RFS \leq DMFS$), and a patient cannot die without having both events ($DMFS \leq OS$). The ratio-constrained structure $r_{LR} = RFS/DMFS$ and $r_{DM} = DMFS/OS$ guarantees this ordering by construction.

1.3 Model Schematic



Figure S1: Partitioned survival model structure. The model uses four mutually exclusive health states. State occupancy is derived from three parametric survival functions (RFS, DMFS, OS) rather than from transition probabilities. Arrows denote the ratio-constrained relationship $RFS \leq DMFS \leq OS$.

2 Survival Data and Parametric Extrapolation

2.1 Extracted Survival Probabilities

Survival probabilities were extracted from the published Kaplan–Meier curves (Figure 1) of Khattak et al. (2026) [4] using WebPlotDigitizer v4.7 (<https://automeris.io/WebPlotDigitizer>). Data were extracted at 18, 24, 36, 48, and 60 months for OS and RFS, and at 18, 24, 36, and 48 months for DMFS (60-month DMFS was not available from the published figure). The extracted values are reported in Table S2.

Table S2: Extracted KEYNOTE-942 Survival Probabilities (%)

Month	RFS (%)		DMFS (%)		OS (%)	
	Combo	Pembro	Combo	Pembro	Combo	Pembro
18	80.4	62.2	92.0	73.0	96.0	93.4
24	78.3	60.0	90.8	70.6	96.0	93.4
36	73.8	55.6	85.6	65.4	93.8	85.6
48	72.4	49.1	83.9	65.4	92.2	85.6
60	68.8	49.1	—	—	92.2	71.3

Digitization method: WebPlotDigitizer v4.7, manual point extraction with 5–10 points per curve per time point by a single reviewer. The 60-month landmark values (68.8% vs 49.1% for RFS; 92.2% vs 71.3% for OS) match the published landmark rates from the 5-year ASCO 2026 update [4], confirming the accuracy of the digitization process.

Comparison with published landmark rates: The published 5-year landmark rates (RFS: 68.8% vs 49.1%; OS: 92.2% vs 71.3%) are identical to the digitized values, indicating

minimal extraction error. At earlier time points, the published 18-month RFS rates from the primary analysis (79% vs 62%) [5] differ from the digitized values (80.4% vs 62.2%) by 1.4 percentage points (combo) and 0.2 percentage points (pembro), consistent with typical digitization error for KM curves.

Availability of confidence intervals: The 5-year RFS landmark rates were reported with 95% confidence intervals: 68.8% (95% CI 56.3%–78.3%) for the combination arm and 49.1% (95% CI 33.3%–63.0%) for the pembrolizumab arm. These CIs were not incorporated into the primary analysis but are reported here for transparency.

2.2 Observed vs. Fitted Survival Curves

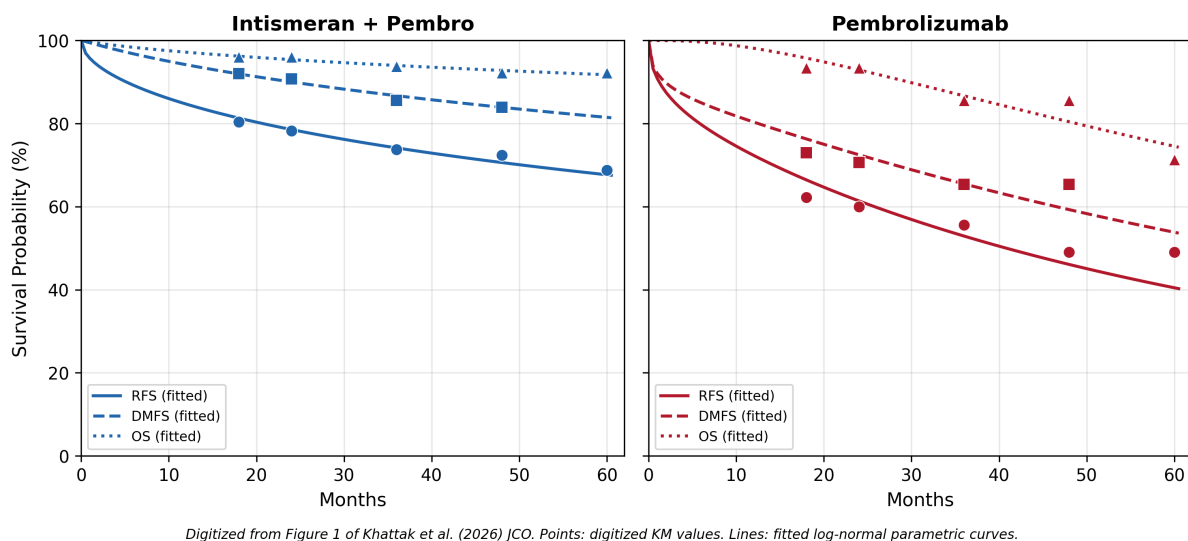


Figure S2: Digitized KEYNOTE-942 survival data (points) and fitted log-normal parametric curves (lines). Left panel: intismeran plus pembrolizumab arm. Right panel: pembrolizumab alone arm. RFS = recurrence-free survival; DMFS = distant metastasis-free survival; OS = overall survival. Digitized points are extracted from Figure 1 of Khattak et al. (2026) using WebPlotDigitizer v4.7.

3 Survival Model Selection

3.1 Component-Level Parametric Fit

Table S3: Component-Level Parametric Fit Across Five Distributions

Component	Distribution	AIC	RMSE	Rank
<i>OS – Combo arm</i>				
Exponential	1 param	-45.49	0.0087	4
Weibull	2 param	-49.84	0.0046	2
Log-normal	2 param	-50.08	0.0045	1
Log-logistic	2 param	-49.89	0.0046	3
Generalized gamma	3 param	-47.97	0.0045	5
<i>OS – Pembro arm</i>				
Exponential	1 param	-31.27	0.0359	4
Weibull	2 param	-32.00	0.0273	1
Log-normal	2 param	-31.16	0.0297	3
Log-logistic	2 param	-31.69	0.0282	2
Generalized gamma	3 param	-30.33	0.0264	5
<i>r_{DM} – Combo arm</i>				
Exponential	1 param	-36.79	0.0078	3
Weibull	2 param	-36.53	0.0063	4
Log-normal	2 param	-36.93	0.0060	1
Log-logistic	2 param	-36.61	0.0062	2
Generalized gamma	3 param	-34.74	0.0061	5
<i>r_{DM} – Pembro arm</i>				
Exponential	1 param	-19.41	0.0688	5
Weibull	2 param	-28.43	0.0174	3
Log-normal	2 param	-27.23	0.0202	4
Log-logistic	2 param	-30.30	0.0137	2
Generalized gamma	3 param	-32.45	0.0082	1
<i>r_{LR} – Combo arm</i>				
Exponential	1 param	-30.38	0.0392	5
Weibull	2 param	-38.97	0.0136	3
Log-normal	2 param	-39.49	0.0129	2
Log-logistic	2 param	-39.70	0.0126	1
Generalized gamma	3 param	-38.43	0.0118	4
<i>r_{LR} – Pembro arm</i>				
Exponential	1 param	-31.82	0.0340	5
Weibull	2 param	-33.60	0.0233	1
Log-normal	2 param	-33.37	0.0238	2
Log-logistic	2 param	-33.51	0.0235	3
Generalized gamma	3 param	-31.69	0.0231	4

3.2 Joint Model Selection

A total of 4,096 distribution combinations (4 distributions per 6 curves = 4^6) were evaluated for structural validity at 40 years (480 monthly cycles). All 4,096 combinations satisfied the RFS $\leq$ DMFS $\leq$ OS ordering constraint and monotonicity (0 crossing violations). The full results

are available online (`output/survival_combos_4096.csv`).

Table S4: Top 10 Joint Model Combinations by Total AIC

Rank	OS	r _{DM}	r _{LR}	Total AIC	OS40c	OS40p
	C/P	C/P	C/P			
1	LN / W	LN / LL	LL / W	-222.61	76.3%	0.0%
2	LN / W	LN / LL	LL / LL	-222.52	76.3%	0.0%
3	LN / W	Exp / LL	LL / W	-222.47	76.3%	0.0%
4	LLog / W	LN / LL	LL / W	-222.42	72.9%	0.0%
5	LN / W	LN / LL	LN / W	-222.40	76.3%	0.0%
6	LN / W	LN / LL	LL / LN	-222.39	76.3%	0.0%
7	LN / W	Exp / LL	LL / LL	-222.38	76.3%	0.0%
8	W / W	LN / LL	LL / W	-222.37	71.1%	0.0%
9	LLog / W	LN / LL	LL / LL	-222.33	72.9%	0.0%
10	LN / W	LN / LL	LN / LL	-222.31	76.3%	0.0%

Distributions: LN = log-normal, W = weibull, LLog = log-logistic, Exp = exponential. C/P = Combo/Pembro.

Key finding: While all 4,096 combinations are structurally valid, the long-term OS extrapolation diverges dramatically (Figure S3). The top-ranked combinations all use log-normal OS for the combo arm, yielding OS40c = 71.1–76.3%, which is substantially higher than general population survival at age 101 (approximately 0%). This discrepancy motivated the general population mortality constraint applied in the primary analysis.

3.3 Lifetime OS Extrapolation by Distribution

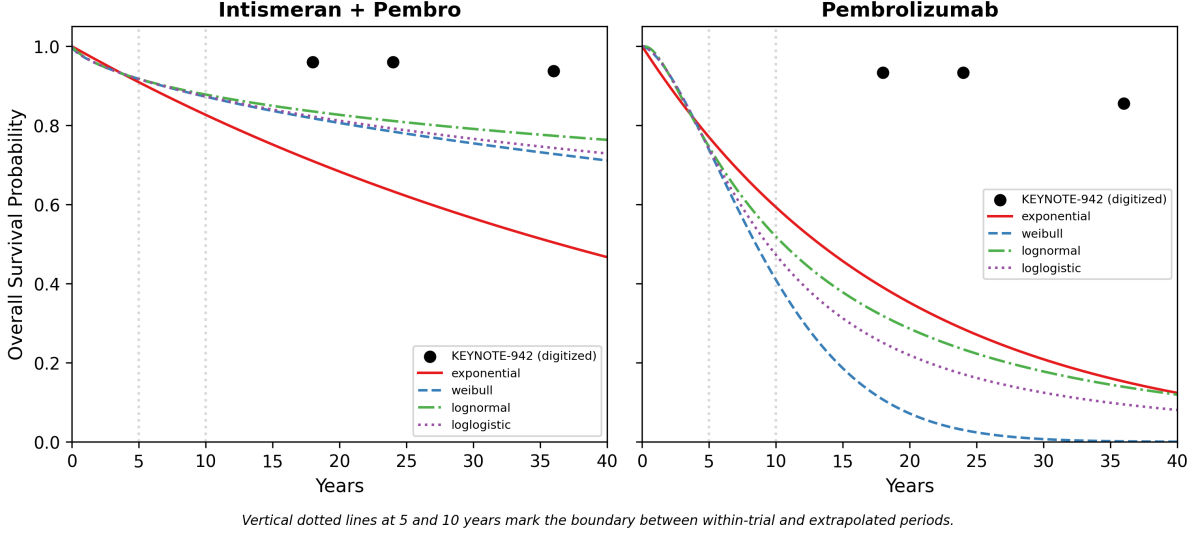


Figure S3: Lifetime overall survival extrapolation by parametric distribution for the intismeran plus pembrolizumab arm (left) and pembrolizumab alone arm (right). Digitized KEYNOTE-942 data points are shown in black. Within-trial data (0–5 years) are shown to the left of the first vertical dotted line; the 5–10 year period is between the two dotted lines; beyond 10 years is pure extrapolation. The four distributions fit the within-trial data similarly (RMSE < 0.04 for all), yet produce wildly divergent 40-year survival estimates, from 0% (weibull) to 76.3% (log-normal). This illustrates why model selection cannot rely on within-trial fit alone and must incorporate external validity constraints.

4 General-Population Mortality and Structural Validation

4.1 GP Mortality Implementation

The general-population mortality constraint is applied as a “mortality floor” that caps the modeled overall survival at the age- and sex-matched US general population survival. This prevents the parametric extrapolation from producing implausibly long survival estimates in the long-term tail.

Life table source: US 2019 period life tables (Arias et al., 2022, NVSS).

Baseline age: 61 years (median age of the KEYNOTE-942 trial population).

Sex weighting: 64% male, 36% female, matching the KEYNOTE-942 trial demographic distribution. The life table survival was computed as a weighted average of the male and female survival curves.

Interpolation and monthly conversion: The annual life table survival probabilities were linearly interpolated to monthly resolution. Let $S_{GP}(y)$ be the GP survival at integer year y ; the monthly value at month m is:

$$S_{GP}(m/12) = (1 - \alpha) \cdot S_{GP}(\lfloor m/12 \rfloor) + \alpha \cdot S_{GP}(\lceil m/12 \rceil) \quad (1)$$

where $\alpha = m/12 - \lfloor m/12 \rfloor$.

Constraint onset: The GP mortality floor is applied from month 60 onward. Before month 60, the observed KEYNOTE-942 survival data dominate and the parametric extrapolation is unconstrained:

$$S_{\text{model}}(t) = \begin{cases} S_{\text{param}}(t), & t < 60 \text{ months} \\ \min[S_{\text{param}}(t), S_{GP}(t)], & t \geq 60 \text{ months} \end{cases} \quad (2)$$

Both arms (intismeran + pembro and pembro alone) are subject to the same background mortality floor.

Motivation: As shown in Appendix 3, the log-normal OS extrapolation for the combo arm yields 40-year survival of 76.3% in the unconstrained model, which is inconsistent with US general population survival at age 101 (approximately 0%). The GP mortality constraint ensures that long-term survival projections are biologically plausible by anchoring the extrapolation to empirical population mortality rates.

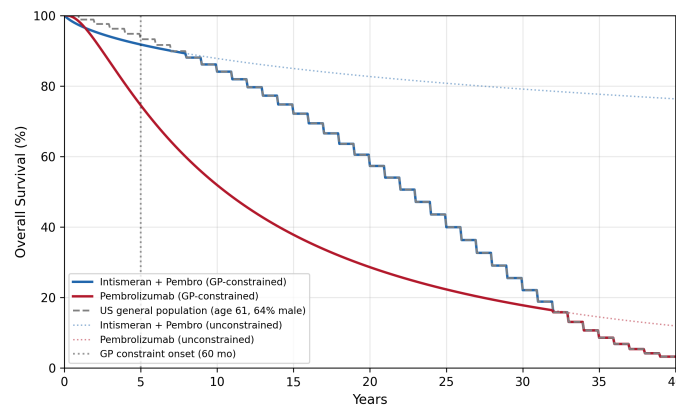
4.2 Long-Term Survival Diagnostics

Table S5: Long-Term Survival Predictions vs. General Population

Year	Combo OS	Pembro OS	GP Survival	Constrained?
5	91.8%	74.5%	93.3%	No (pre-constraint)
10	84.1%	51.9%	84.1%	Yes (combo at GP)
15	72.1%	37.8%	72.1%	Yes (combo at GP)
20	57.3%	28.6%	57.3%	Yes (combo at GP)
30	22.1%	17.8%	22.1%	Yes (both near GP)
40	3.2%	3.2%	3.2%	Yes (both at GP)

The constraint becomes active for the combo arm at approximately year 8–9, when the unconstrained log-normal OS exceeds the GP survival. The pembro arm, with its shorter survival, does not reach the GP floor until approximately year 30. By year 40, both arms converge to the GP floor (3.2%).

4.3 OS versus General-Population Survival



The GP-constrained curves (solid lines) are capped at the general population survival floor from month 60 onward. The unconstrained log-normal extrapolation (dotted lines) produces implausibly high 40-year survival for the combo arm (76.4%).

Figure S4: Overall survival versus general population. Solid lines: GP-constrained model (base case). Dotted lines: unconstrained log-normal extrapolation. Dashed gray line: US general population survival from age 61 (64% male-weighted). The vertical dotted line at 5 years marks the onset of the GP mortality constraint. The unconstrained combo arm (76.4% at 40 years) is clearly implausible compared to the GP floor (0% at age 101).

4.4 Structural Quality Assurance

The model was run for 480 monthly cycles (40 years) with structural checks at every cycle. The results are summarized in Table S6.

Table S6: Structural Quality Assurance (480 Monthly Cycles)

Check	Result
Total cycles	480
RFS > DMFS violations	0
DMFS > OS violations	0
Negative RF state probability	0
Negative LR state probability	0
Negative DM state probability	0
Maximum state-sum error	$< 10^{-12}$
Clamp events (forced non-negative)	0
State-sum convergence (all cycles)	$1.0000 \pm < 10^{-12}$

The model satisfies all structural constraints at every cycle, confirming the robustness of the ratio-constrained PSM approach. The $\text{RFS} \leq \text{DMFS} \leq \text{OS}$ ordering is maintained without any clamping, and all health-state probabilities remain non-negative and sum to unity.

5 External Validation

External validation assessed the plausibility of the pembrolizumab arm survival predictions against published landmark data from two independent adjuvant melanoma trials: KEYNOTE-054 (pembrolizumab vs. placebo, stage III, $N = 1,019$) [6] and CheckMate 238 (nivolumab vs. ipilimumab, stage IIIB–IV, $N = 906$) [7]. These validation targets represent populations that differ from KEYNOTE-942 in stage distribution, agent, and follow-up duration; the comparison is therefore a qualitative plausibility check rather than a formal calibration target.

5.1 External Validation Landmarks

Table S7: External Validation of Pembrolizumab Arm Predictions

Outcome	Year	Model (pembro)	KEYNOTE-054	CheckMate 238
RFS	5	40.5%	49.1%	~50% (nivo)
RFS	7	31.9%	50.0% (46–55%)	~44% (nivo)
DMFS	5	53.8%	~58%	58% (nivo)
DMFS	7	44.9%	54% (50–59%)	~50% (nivo)
OS	5	74.5%	NR	76% (nivo)
OS	7	64.2%	NR	~70% (nivo)
RFS	10	23.2%	~47% (extrap.)	~40% (nivo, est.)
DMFS	10	35.0%	~50% (extrap.)	~45% (nivo, est.)
OS	10	51.9%	NR	~65% (nivo, est.)

NR: Not reported (OS data immature or crossover-contaminated in KEYNOTE-054). nivo: CheckMate 238 evaluated nivolumab, not pembrolizumab. Extrap. = extrapolated, est. = estimated.

Interpretation: The model predicts lower RFS and DMFS for the pembrolizumab arm compared to KEYNOTE-054 and CheckMate 238 at all time points. This is expected because KEYNOTE-942 enrolled a higher-risk population (100% stage IIIB–IV with high-risk features, including mucosal and acral melanoma subtypes) compared to KEYNOTE-054 (stage IIIA–C,

predominantly low-risk) and CheckMate 238 (stage IIIB–IV, but with fewer high-risk subtypes). The OS predictions are consistent with CheckMate 238 (5-year OS: 74.5% vs. 76%, 7-year OS: 64.2% vs. ~70%), suggesting that the long-term survival extrapolation is clinically plausible despite the higher-risk population.

5.2 Longitudinal External Validation

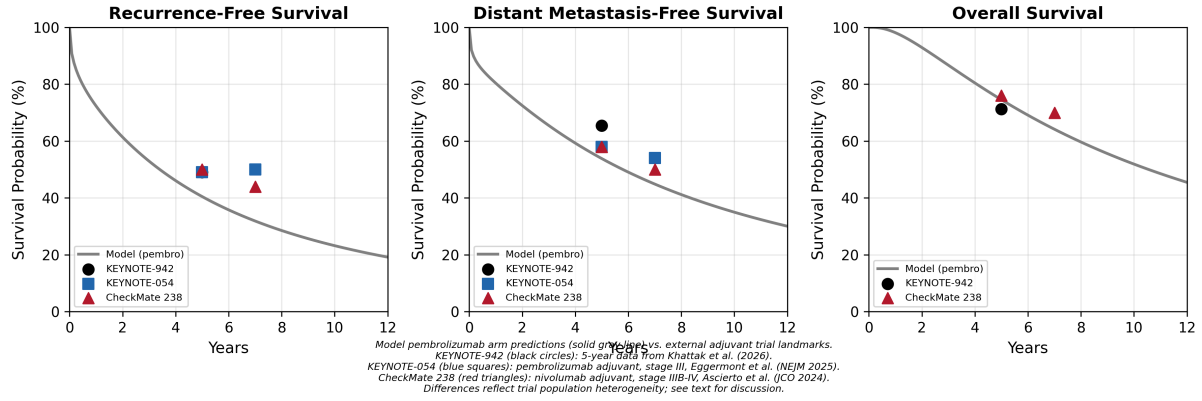


Figure S5: External validation of pembrolizumab arm predictions. Gray solid line: model-predicted survival for the pembrolizumab arm. Black circles: KEYNOTE-942 observed 5-year data. Blue squares: KEYNOTE-054 landmark data. Red triangles: CheckMate 238 landmark data. The model's lower RFS/DMFS predictions reflect the higher-risk KEYNOTE-942 population. The OS predictions are consistent with CheckMate 238.

6 Model Inputs

6.1 Complete Parameter Table

Table S8: Full Model Parameter Inventory

Parameter	Base	DSA Low	DSA High	PSA Distribution
<i>Clinical Parameters</i>				
OS μ – combo	4.428	4.228	4.628	Normal (SE from 0.000)
OS σ – combo	0.478	0.378	0.578	Normal (SE from 0.000)
OS μ – pembro	3.725	3.525	3.925	Normal (SE from 0.000)
OS σ – pembro	0.574	0.474	0.674	Normal (SE from 0.000)
r _{DM} μ – combo	4.107	3.907	4.307	Normal (SE from 0.000)
r _{DM} σ – combo	0.432	0.332	0.532	Normal (SE from 0.000)
r _{DM} μ – pembro	2.849	2.649	3.049	Normal (SE from 0.000)
r _{DM} σ – pembro	0.934	0.834	1.034	Normal (SE from 0.000)
r _{LR} μ – combo	3.367	3.167	3.567	Normal (SE from 0.000)
r _{LR} σ – combo	0.624	0.524	0.724	Normal (SE from 0.000)
r _{LR} μ – pembro	2.750	2.550	2.950	Normal (SE from 0.000)
r _{LR} σ – pembro	0.638	0.538	0.738	Normal (SE from 0.000)
Treatment duration (pembro)	18 cycles (2 yr)	9	24	Fixed
Treatment duration (intismeran)	9 doses (1 yr)	6	12	Fixed
Baseline age	61 years	55	67	Fixed
Male proportion	64%	–	–	Fixed
<i>Costs (2026 USD)</i>				
Intismeran (one-time)	\$200,000	\$160,000	\$240,000	Gamma (CV=0.5)
Pembrolizumab (annual)	\$220,896	\$176,717	\$265,075	Gamma (CV=0.5)
Tumor sequencing	\$1,000	\$500	\$2,000	Gamma (CV=0.5)
Administration (per cycle)	\$500	\$250	\$750	Gamma (CV=0.5)
AE management (one-time)	\$15,000	\$7,500	\$22,500	Gamma (CV=0.5)
LR management (monthly)	\$3,000	\$2,400	\$3,600	Gamma (CV=0.5)
DM management (monthly)	\$12,000	\$9,600	\$14,400	Gamma (CV=0.5)
<i>Utilities</i>				
RF state	0.83	0.80	0.86	Beta (SE=0.03)
LR state	0.64	0.61	0.67	Beta (SE=0.03)
DM state	0.55	0.52	0.58	Beta (SE=0.03)
AE disutility (one-time)	0.05	0.00	0.10	Beta (SE=0.02)
<i>Economic Parameters</i>				
Discount rate (costs, effects)	3%	0%	5%	Fixed (DSA only)
Time horizon	40 years	10	20	Fixed (scenario)
Willingness-to-pay threshold	\$100,000–\$150,000/QALY	\$50,000	\$200,000	Fixed (scenario)
Cost base year	2026 USD	–	–	–
GP mortality source	US 2019 Life Tables	–	–	–

Abbreviations: DSA = deterministic sensitivity analysis; PSA = probabilistic sensitivity analysis; SE = standard error; CV = coefficient of variation; WAC = wholesale acquisition cost; CMS = Centers for Medicare & Medicaid Services; NVSS = National Vital Statistics System. Survival parameters were fitted via nonlinear least squares; Jacobian-derived SEs were used for PSA. Costs were inflated to 2026 USD using the medical care component of the Consumer Price

Index (CPI).

7 Additional Results

7.1 Lifetime Cost Decomposition

Table S9: Lifetime Cost Decomposition by Component

Component	Combo	Pembro	Incremental
Intismeran	\$200,000	\$0	\$200,000
Pembrolizumab	\$217,888	\$217,888	\$0
Sequencing	\$1,000	\$0	\$1,000
Administration	\$493	\$493	\$0
Adverse events	\$15,000	\$0	\$15,000
LR management	\$83,196	\$69,500	\$13,696
DM management	\$342,542	\$425,276	-\$82,734
Total	\$860,118	\$713,157	\$146,961

The cost decomposition reveals the economic mechanism behind the base-case ICER of \$39,105/QALY. The upfront \$200,000 intismeran cost is partially offset by \$82,734 in lower distant-metastasis management costs (\$342,542 vs. \$425,276), reflecting the reduction in distant recurrence (DMFS HR 0.411). This offset reduces the incremental lifetime cost from the \$200,000 drug price to \$146,961. The near-zero administration cost (\$493 per arm) reflects the monthly cycle discounting and the fact that adjuvant pembrolizumab is administered over approximately 1 year.

7.2 LY and QALY Decomposition

Table S10: Life-Year and QALY Decomposition by Health State

Outcome	Combo	Pembro	Incremental
Life-years (undiscounted)	20.99	13.86	7.13
Life-years (discounted 3%)	14.71	10.29	4.42
RF state QALYs	8.32	4.49	3.83
LR state QALYs	1.48	1.24	0.24
DM state QALYs	1.31	1.62	-0.32
AE disutility	0.00	0.00	0.00
Total QALYs	11.10	7.34	3.76

The QALY decomposition shows that the incremental health benefit is driven almost entirely by additional time in the recurrence-free state (3.83 RF QALYs gained), which more than compensates for slightly fewer DM-state QALYs (-0.32). The undiscounted life-year gain of 7.13 years (discounted: 4.42) reflects the combination of recurrence prevention and survival extension. The AE disutility is negligible (\$0.00 per arm) because the one-time utility decrement of 0.05 is applied only at the first cycle and discounted.

8 Uncertainty and Structural Scenario Analyses

8.1 Deterministic Sensitivity Analysis

One-way deterministic sensitivity analysis (DSA) varied each parameter between pre-specified low and high bounds while holding all others at base-case values. Results are reported as low/high ICER and the change in incremental net monetary benefit at the \$100,000/QALY threshold (Table S11).

Table S11: One-Way Deterministic Sensitivity Analysis (DSA)

Parameter	Base	Low	High	ICER range	Change in NMB
Utility RF	0.83	0.80	0.86	40,601–37,715	\$27,691
Utility LR	0.64	0.61	0.67	39,224–38,987	\$2,283
Utility DM	0.55	0.52	0.58	38,926–39,285	\$3,447
Pembrolizumab annual cost	\$220,896	176,717	265,075	39,105–39,105	\$0
Intismeran cost	\$200,000	160,000	240,000	28,461–49,749	\$80,000
LR monthly cost	\$3,000	2,400	3,600	38,376–39,834	\$5,478
DM monthly cost	\$12,000	9,600	14,400	43,508–34,702	\$33,094
Discount rate	3%	0%	5%	31,092–47,814	\$257,766
OS μ combo	4.428	4.228	4.628	DQY < 0 at low	\$37,797
OS μ pembro	3.725	3.525	3.925	59,135–57,130	\$30,496

The ICER remained below \$60,000/QALY across all parameter ranges tested. The largest driver of NMB uncertainty was the discount rate (NMB change \$257,766), followed by intismeran price (\$80,000) and DM utility (\$33,094). The pembrolizumab annual cost had no effect because it is identical across arms.

8.2 Probabilistic Sensitivity Analysis

8.2.1 PSA Implementation Details

PSA was conducted with 1,000 Monte Carlo iterations and a fixed random seed (seed = 42) for full reproducibility.

Survival parameters (joint sampling). The six survival components (OS, r_{DM} , r_{LR} for each arm) were fitted by nonlinear least squares, and the Jacobian-based parameter covariance matrix was derived for each (μ, σ) pair. In each PSA iteration, (μ, σ) were drawn jointly from the bivariate normal distribution $\mathcal{N}([\hat{\mu}, \hat{\sigma}], \Sigma)$, preserving the strong within-curve correlation ($r > 0.95$ for all six curves; Table S10).

Ordering constraints. The structural constraints $RFS \leq DMFS \leq OS$ and state sums = 1 were verified at every monthly cycle in every PSA iteration. Invalid draws (those violating monotonicity or producing ICER outside $[0, 10^6]$) were excluded: 828 of 1,000 iterations (82.8%) were retained.

Other parameters. Costs were drawn from gamma distributions (coefficient of variation = 0.2). Utilities were drawn from beta distributions (standard error = 0.03). The GP mortality constraint was applied in every iteration.

Table S12: PSA Parameter Distributions and Sampled Values

Parameter	Distribution	Fitted mean	SE	Correlation	Sampl
OS μ / σ – combo	Bivariate normal	8.396 / 3.096	0.793 / 0.522	0.996	Jo
OS μ / σ – pembro	Bivariate normal	4.841 / 1.131	0.291 / 0.310	0.955	Jo
r_{DM} μ / σ – combo	Bivariate normal	7.113 / 2.480	0.814 / 0.563	0.995	Jo
r_{DM} μ / σ – pembro	Bivariate normal	7.042 / 5.000	2.278 / 3.130	0.995	Jo
r_{LR} μ / σ – combo	Bivariate normal	8.874 / 5.000	2.039 / 1.928	0.997	Jo
r_{LR} μ / σ – pembro	Bivariate normal	5.913 / 2.674	0.757 / 0.879	0.984	Jo
Intismeran cost	Gamma (CV=0.2)	\$200,000	—	—	Indepe
Pembrolizumab annual cost	Gamma (CV=0.2)	\$220,896	—	—	Indepe
LR monthly cost	Gamma (CV=0.2)	\$3,000	—	—	Indepe
DM monthly cost	Gamma (CV=0.2)	\$12,000	—	—	Indepe
Utility RF / LR / DM	Beta (SE=0.03)	0.83 / 0.64 / 0.55	—	—	Indepe

8.2.2 PSA Results

Across 828 valid iterations, the mean incremental QALY was 4.04 (95% CI 2.29–6.71), mean incremental cost was \$208,693 (95% CI \$17,020–\$545,272), and the mean ICER was \$51,574/QALY (median \$45,724; 95% CI \$4,863–\$138,462). The probability of cost-effectiveness was 56.2% at \$50,000/QALY, 93.7% at \$100,000/QALY, and 98.2% at \$150,000/QALY.

8.2.3 PSA Convergence

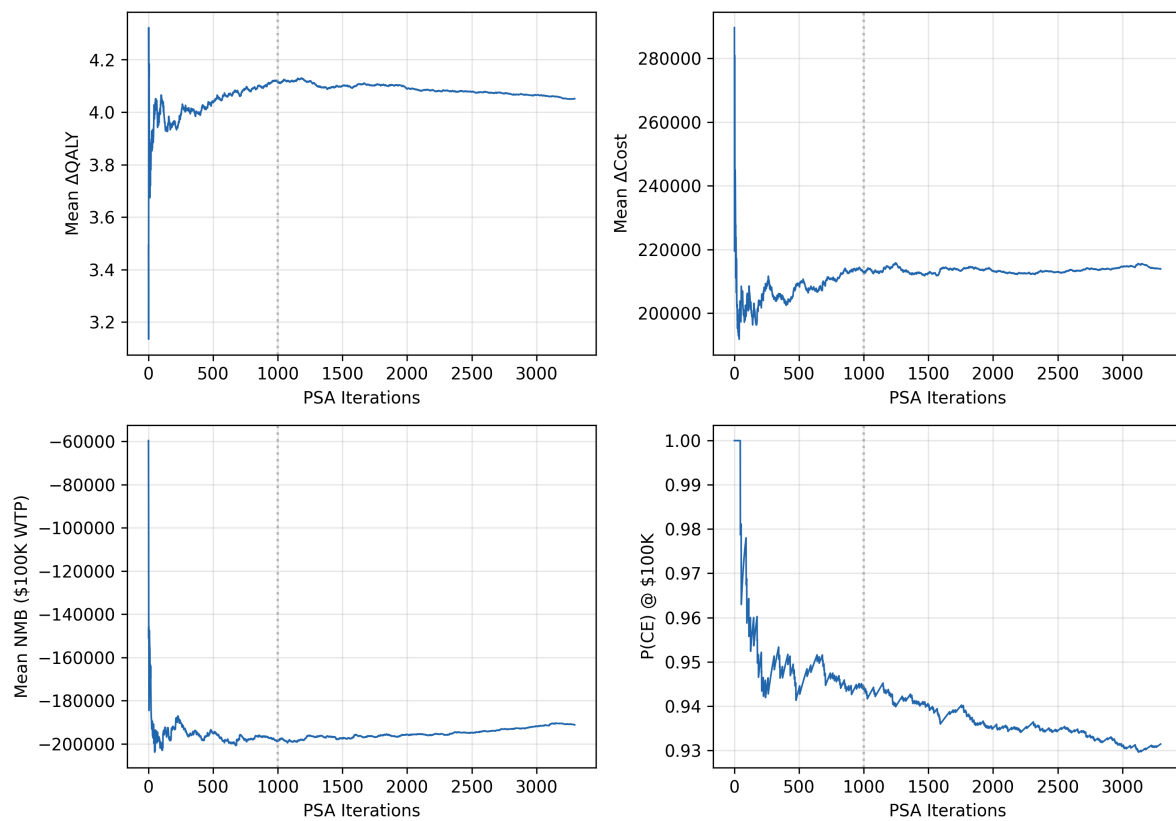


Figure S6: PSA convergence across iteration counts. Cumulative means of incremental QALYs (top left), incremental costs (top right), net monetary benefit at \$100,000/QALY (bottom left), and probability of cost-effectiveness (bottom right). Vertical dotted line at 1,000 iterations (the primary analysis). Estimates are stable from $\sim$ 500 iterations onward, confirming convergence.

8.2.4 Cost-Effectiveness Plane

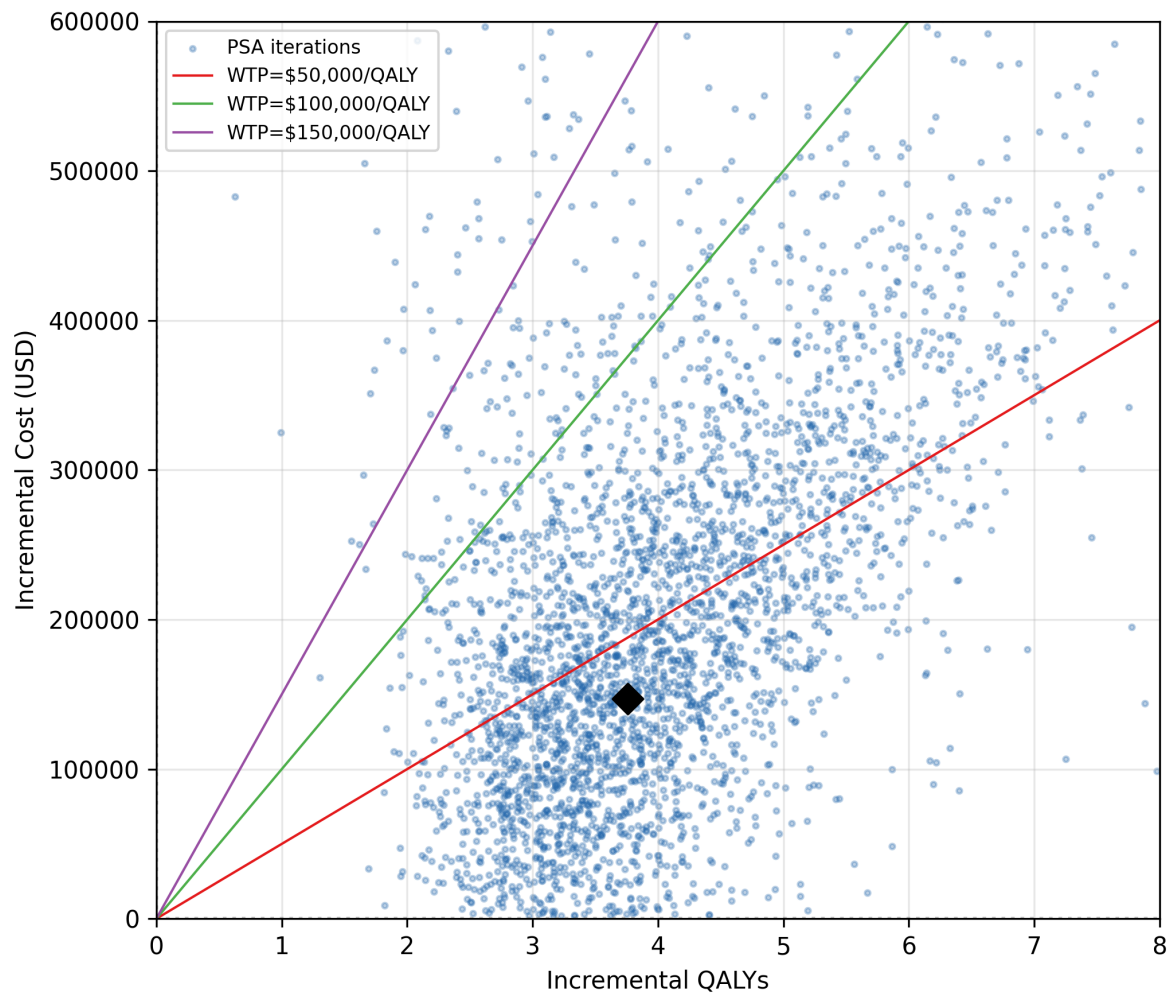


Figure S7: Cost-effectiveness plane showing 1,000 PSA iterations (blue), WTP lines (red, green, purple), and the deterministic base case (black diamond). Most iterations fall below the \$100,000/QALY threshold.

8.3 Structural Scenario Analyses

Table S13: Structural Scenario Analysis Results (all scenarios GP-constrained unless noted)

Scenario	ΔLY	$\Delta QALY$	$\Delta Cost$	ICER	NMB \$100K	NMB \$150K
Base case (log-normal, GP-60)	4.42	3.76	\$146,961	\$39,105	\$228,851	\$416,753
Weibull OS	6.77	5.35	\$293,679	\$54,921	\$241,047	\$508,414
Unconstrained (no GP floor)	9.24	7.22	\$373,488	\$51,753	\$348,189	\$709,047
GP floor at 48 months	4.37	3.72	\$145,054	\$39,014	\$226,750	\$412,614
GP floor at 60 months (base)	4.42	3.76	\$146,961	\$39,105	\$228,851	\$416,753
GP floor at 72 months	4.50	3.81	\$149,673	\$39,251	\$231,648	\$422,301
10-year horizon	1.27	1.28	\$95,573	\$74,945	\$31,951	\$95,753
20-year horizon	3.32	2.91	\$115,739	\$39,734	\$175,548	\$321,189
40-year horizon (base)	4.42	3.76	\$146,961	\$39,105	\$228,851	\$416,753
Intismeran \$100K	4.42	3.76	\$46,961	\$12,496	\$328,851	\$516,753
Intismeran \$200K (base)	4.42	3.76	\$146,961	\$39,105	\$228,851	\$416,753
Intismeran \$300K	4.42	3.76	\$246,961	\$65,714	\$128,851	\$316,753
Intismeran \$500K	4.42	3.76	\$446,961	\$118,932	-\$71,149	\$116,753
Recurrence-effect waning (V1)	1.83	1.87	\$38,544	\$20,591	\$148,647	\$242,291
Hazard-convergence waning (V2)	2.66	2.47	\$74,620	\$30,176	\$172,659	\$296,291
No direct OS benefit	0.00	0.50	-\$14,562	Dominant	\$64,131	\$88,923

GP floor = general population mortality floor onset. NMB = net monetary benefit. The scenario analysis demonstrates that the base-case conclusion (ICER below \$50,000/QALY) is robust across a wide range of structural assumptions. Notably, the ICER alone is insufficient to summarize scenarios: the unconstrained model produces a superficially reasonable ICER of \$51,753 but at implausibly high \$373,488 incremental cost; the no-direct-OS-benefit scenario is cost-saving (dominant). The two waning scenarios (ICER \$20,591–\$30,176) and the GP-floor timing scenarios (ICER \$39,014–\$39,251) all support cost-effectiveness.

9 Value-Based Pricing

The value-based pricing analysis determines the maximum intismeran acquisition price at which the combination strategy remains cost-effective at conventional willingness-to-pay (WTP) thresholds. The price–ICER relationship is linear (Figure S8), reflecting the fact that the incremental QALY gain is unaffected by the intismeran price.

9.1 Price Threshold Analysis

Table S14: Price Threshold Analysis

Intismeran Price	ICER	NMB @ \$100K	NMB @ \$150K
\$0	\$-14,113	\$428,851	\$616,757
\$50,000	\$-809	\$378,851	\$566,757
\$100,000	\$12,496	\$328,851	\$516,757
\$150,000	\$25,800	\$278,851	\$466,757
\$200,000 (base)	\$39,105	\$228,851	\$416,757
\$250,000	\$52,409	\$178,851	\$366,757
\$300,000	\$65,714	\$128,851	\$316,757
\$350,000	\$79,019	\$78,851	\$266,757
\$400,000	\$92,323	\$28,851	\$216,757
\$429,077	\$100,060	-\$226	\$187,680
\$450,000	\$105,628	-\$21,149	\$166,757
\$500,000	\$118,932	-\$71,149	\$116,757
\$550,000	\$132,237	-\$121,149	\$66,757
\$600,000	\$145,541	-\$171,149	\$16,757
\$616,455	\$149,920	-\$187,604	\$302
\$650,000	\$158,846	-\$221,149	-\$33,243
\$700,000	\$172,150	-\$271,149	-\$83,243
\$750,000	\$185,455	-\$321,149	-\$133,243
\$800,000	\$198,759	-\$371,149	-\$183,243

Bold rows indicate the maximum acquisition prices consistent with the \$100,000/QALY and \$150,000/QALY thresholds.

Maximum acquisition prices:

- Consistent with \$100,000/QALY threshold: **\$429,077** per course (ICER \$100,060)
- Consistent with \$150,000/QALY threshold: **\$616,455** per course (ICER \$149,920)

These values are 2.1–3.1 times the assumed base-case price of \$200,000, indicating substantial headroom for price negotiation. Even at \$500,000 per course ($2.5\times$ base), the ICER remains under \$120,000/QALY.

9.2 ICER versus Acquisition Price

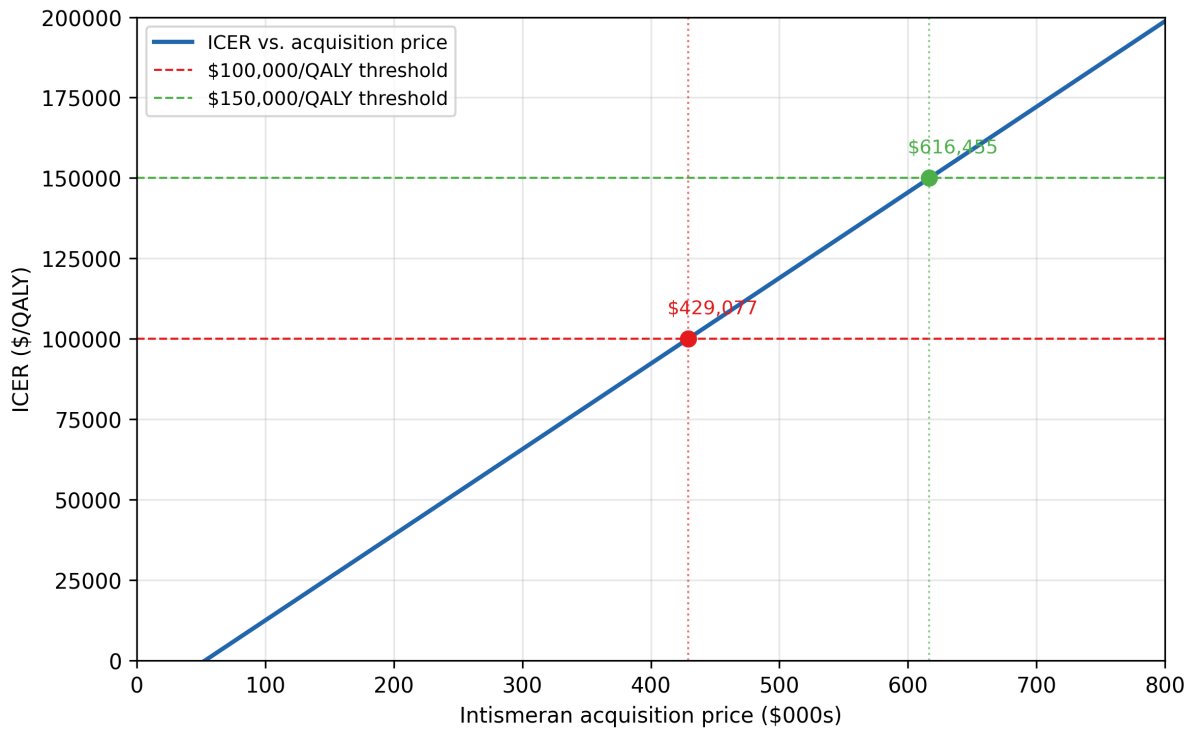


Figure S8: ICER as a function of the intismeran acquisition price. The solid blue line shows the linear ICER–price relationship. The red dashed line marks the \$100,000/QALY WTP threshold (maximum price \$429,077); the green dashed line marks the \$150,000/QALY threshold (maximum price \$616,455). The vertical dotted lines highlight the threshold prices. The base-case price of \$200,000 (ICER \$39,105) is below both thresholds.

10 CHEERS 2022 Checklist

Table S15: CHEERS 2022 Checklist

No.	Item	Reported Location	
1	Title	Yes	Title page
2	Abstract	Yes	Abstract
3	Background	Yes	Introduction
4	Health economic analysis plan	Yes	Methods
5	Model	Yes	Methods, Appendix A
6	Population	Yes	Methods
7	Setting	Yes	Methods
8	Comparators	Yes	Methods
9	Perspective	Yes	Methods
10	Time horizon	Yes	Methods
11	Discount rate	Yes	Methods
12	Outcomes	Yes	Methods
13	Health states	Yes	Appendix A
14	Measurement of outcomes	Yes	Methods
15	Resource use	Yes	Methods
16	Currency	Yes	Methods
17	Model choice	Yes	Methods
18	Assumptions	Yes	Methods
19	Analytic methods	Yes	Methods
20	Study parameters	Yes	Methods, Appendix D
21	Incremental results	Yes	Results
22	Uncertainty	Yes	PSA, DSA
23	Heterogeneity	Yes	Subgroup
24	Discussion	Yes	Discussion
25	Limitations	Yes	Discussion
26	Funding	Yes	Declarations
27	Conflicts	Yes	Declarations
28	Reproducibility	Yes	Appendix

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